

Personal Organizational Plan

Name: Sajjan Gautam

Centennial College

Placement Facility: 5E Rehab Unit – Centenary Site

Clinical Instructor: Denise Lacey

Date: 2026/02/13

PERSONAL ORGANIZATIONAL PLAN (POP)

Students complete a POP for each new client and update existing POP's weekly

STUDENT: Sajjan Gautam WK# 6 DATE: 2026/02/13

CARE PARTNER: **Abvie Gaile Aviles** NURSING SUPERVISOR: **Denise Lacey**

LEVEL OF CARE

Code	Patient's/resident's/client's self-evaluation
0	Independent-Able to accomplish all ADLs without intervention from staff or family: bathe, dress, groom, transfer and ambulate, toilet, communicate
1	Minimal Assist- Walks/transfers independently; infrequent falls – verbal reminders.
2	Stand-By Assist-Transfer/stand-by assist may be needed. ADLs – reminders, verbal cues. Occasional incontinence assistance. Bathing set-up helpful.
3	Hands-On-Transfer-one person assist usually needed. ADLs – verbal cues to hands-on assist. Incontinence mgmt. Bathing assistance needed.
4	Total Assist-Transfer-mechanical lift/two-person transfer/bedfast. ADLs-hands-on assistance. Incontinence mgmt. Bathing assistance.

Client No. (Initial & Room No.)	Health Problem / Diagnosis	Level of Care
1. C.K. 508-01	Arthritis, Hypertension, Cholangitis, Ascending Cholangitis, Atrial Fibrillation, CHF (Congestive Heart Failure), Systemic Inflammatory Response Syndrome	3
2. C.O.B 508-02	Arthritis, CTIs (Carpal Tunnel Syndrome), Hypertension, Seasonal Allergies, sickle cell trait, Anxiety	0
3. D.L.N. 502-01	Bronchiectasis, Cerebral venous sinus thrombosis, CKD (chronic kidney disease), Depression, History of Chemotherapy, Hypertension, Intracranial Mass, Overactive Bladder, Uterine Cancer, Sepsis, Endometrial Carcinoma	2
4. A.B.M. 536-02	Anxiety associated with depression, COPD, BPH (Benign Prostatic Hyperplasia), GERD celiac disease, Chronic back-pain, MDD, Hypertension, High cholesterol	1
5. B.D. 536-01	Depression, Diabetes, Mellitus, High Cholesterol, Hypertension	3

TIME	PLANS/ACTIVITIES TO MEET THE CLIENT NEEDS	OBSERVATIONS TO REPORT AND CHART
0645	Reached SHN site in street clothes, Wore scrubs ready for shift	
0700	Pre-conference with clinical instructor to discuss client care plan, safety, mobility, transfer, behavior and so on	
0730	Shift Started Client 1: Safety check completed with bed in lowest position, brakes locked, and call bell within reach. Morning hygiene provided	Client asked for brief change. Assisted with brief and linens change. Oral care assistance

	including face cleansing and perineal care. Repositioned using pillows to support back and head and to reduce pressure. Client 2: Safety check performed, bed in the lowest position, call bell in reach and environment cleared of hazards. Morning care and oral care done by client no assistance needed. Client 3: Safety check performed, bed in the lowest position, call bell within reach and environment free of hazards. Client complained of sharp ear pain.	provided. No other concerns noted. Client is completely independent. Assistance provided with small chores like providing hot water and cleaning the table. No concerns noted. Client complained of sharp ear pain. Notified assigned nurse. Preferred to do oral care after
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0830 0910	Client 1: Client wanted shower so assisted with shower. Client appeared happy with shower. Client 2: Resting comfortably in bed, watching news on her phone. No assistance needed with anything. Client 3: Assisted with transfer from bed to chair. Client 1: Completed hand hygiene and positioned upright in	breakfast. No other concerns noted. Client tolerated well. No concerns noted. No Concerns noted Client tolerated the transfer well. No other concerns noted.
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0930	preparation for breakfast. Client 2: Client sat in a sitting position in bed read for breakfast. Client 3: Completed hand hygiene and prepared for breakfast. Client 1: Breakfast was provided and the client was monitored in-between the meal for tolerance and safety. Client 2: Breakfast was provided, and the client fed herself	Client was alert, cooperative, and ready to eat. Client was alert, cooperative, and ready to eat. Client still had ear pain, but she was still ready to eat. Client tolerated breakfast well with no signs of choking or swallowing difficult.
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0950	independently without any monitoring or assistance. Client 3: Breakfast was provided and monitored in-between to ensure safe intake. Client 1: Breakfast meal intake and fluid intake were documented in EPIC. Client 2: Breakfast meal intake and fluid intake were documented in EPIC.	No monitoring necessary. Client's intake was fair. Client tolerated breakfast well with no signs of choking or swallowing difficulty. Client 1: Meal intake 100%, Fluid intake 339 ml Client 2: Meal intake 100%, fluid intake 489 ml
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	Client 3: Breakfast meal intake and fluid intake were documented in EPIC.	Client 3: Meal intake 100%, Fluid intake 450ml.
1000 – 1100		
1110	BREAK	
	Client 1: Returned from break and completed a safety and comfort check, ensuring proper positioning and call bell in reach.	Client resting comfortably. No additional assistance needed.
	Client 2: Returned from break and completed a safety and comfort check, ensuring the client's needs were met.	Client resting comfortably. No additional assistance needed.
	Client 3: Returned from break	Client resting comfortably.

1130	and completed a safety and comfort check.	Pain reduced after medication from nurse. No additional assistance needed.
1220	Assisted the PSW in charge Abvie for client V.C.'s partial bed bath and linens change. Client 1: Client was positioned in a straight position and prepared ready for lunch. Client 2: Client was ready and positioned in sitting position for lunch.	Documented daily care on EPIC No signs of discomfort or pressure injury. Verbally asked whether any discomfort. Client said, "No" No concerns

1230	Client 3: Client was ready and transferred to chair for lunch. Client 1: Lunch was provided, and the client was monitored during the meal. Client 2: Lunch was provided, and the client fed herself independently. Client 3: Lunch was provided, and the client ate well.	Pain reduced drastically. Client seemed more talkative. Intake was fair. No other concerns. No concerns Intake was excellent. Client ate everything.
1300		

1330	Client 1: Meal tray was cleared, eating area was cleaned, and intake was documented in EPIC.	Client ate 75% of meal and 450 ml of fluids.
	Client 2: Meal tray was cleared, eating area was cleaned, and intake was documented in EPIC.	Client ate 75% of meal and 625 ml of fluids.
	Client 3: Meal tray was cleared, eating area was cleaned, and intake was documented in EPIC.	Client ate 100% of meal and 589 ml of fluids.
	Client 1: Assisted with repositioning in bed for comfort and pressure relief. Ensured brief was clean and dry and call bell within reach.	No signs of pressure injury noted. Client resting comfortably.

1400 1500	Client 2: Client resting in bed watching phone. Safety check completed and water refreshed. Client 3: Assisted with repositioning in chair and ensured comfortable sitting posture with back support. Assisted other SPSWs with their clients in partial bed bath (1 client), perineal care (2 clients) and small chores around the facility.	No concerns Client reading newspaper. No concerns noted.
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1530	Client 1: Final afternoon comfort check completed and positioning adjusted. Client 2: Final safety check completed and environment left clutter-free. Client 3: Final safety and positioning check completed prior to shift wrap-up. Post Conference with CI. Discussed client status, intake/output documentation, safety measures, transfers completed, behavioural	Client comfortable after repositioning. No redness or skin concerns noted. Client independent and stable. No concerns noted. Client tolerated repositioning well. No other concerns noted. All clients comfortable and stable. All required documentation completed. No
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1600	observations, and overall care provided during the shift. End-of-shift	outstanding safety concerns reported.
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GUIDE – RESIDENT/CLIENT INFORMATION AND NEEDS

The Information can be gathered from the patient/resident/client, family, Care Partner, Faculty Instructor

AGE: 71 Y.O. (F) STAGE OF GROWTH & DEVELOPMENT: Late

Adulthood

BIRTHPLACE: Guyana

ETHNIC/CULTURAL BACKGROUND: Guyanese

CULTURAL/RELIGIOUS: PRACTICES/OBSERVANCES: Christianity but doesn't
eat pork (Against religious belief)

PREVIOUS LIVING ARRANGEMENTS: House/ Apartment

PREVIOUS OCCUPATIONS/ROLES: Retired Factory Worker (Packaging
Company Worker)

FAMILY MEMBERS/FRIENDS/VISITORS: Brother only, (parents died long ago,
never married)

REASON/S FOR BEING IN FACILITY: Client was transferred to SHN –

Centenary Site 5East Rehab Unit due to Cholangitis and congestive heart
failure.

LENGTH OF TIME IN FACILITY: 8 days, Client was admitted on 05/02/2026

ABILITY TO SEE/HEAR/SENSE OF TOUCH/TASTE/SMELL: Client's all senses are working perfectly. No impairments noted

ABILITY TO COMMUNICATE/DEVICES/APPROACH: Client is very weak and have a very fragile body. She uses walker to commute and communicates in clear English.

AWARENESS OF SURROUNDINGS/PLACE/OTHER PERSONS/SAFETY: Client is alert and orientated. She is very much aware of her surroundings and does not have any additional safety precautions other than the standard protocol.

PRESENT FEELINGS & CONCERNS: Client is always smiling and happy. She smiles every time someone visits her either for care or a random visit. Her brother visits often and when he does she does seem a little serious but every

time they talk it feels as they have a very deep relationship. Her mood lightens up when the brother visits.

RE: SELF/CONDITION/PLACEMENT/FAMILY/ACTIVITIES/LIFE/COMMUNITY/
etc.

Client is weak and fragile with a history of arthritis, CHF and hypertension.

Client has a brother who is her only family member as she never married, and her parents died when she was very young. The brother was the one who took care of her since and still is, so she has a very deep relationship with the brother.

OVERALL APPEARANCE & ABILITIES/LIMITATIONS TO FUNCTION:

Client is alert, oriented and appears to be very neat and clean. She likes cleanliness. She wants to shower every now and then. She appreciates with any assistance provided and doesn't have any physical limitations other than

the weak and fragile body. She uses a walker to commute, and she is assist x1 with extensive help needed in toileting, ADLs and so on.

Data Sheet for Client 2

GUIDE – RESIDENT/CLIENT INFORMATION AND NEEDS

The Information can be gathered from the patient/resident/client, family, Care Partner, Faculty Instructor

AGE: 67 Y.O. (F) STAGE OF GROWTH & DEVELOPMENT: Middle

Adulthood

BIRTHPLACE: Jamaica

ETHNIC/CULTURAL BACKGROUND: Jamaican-Canadian (Born in Jamaica but raised in Canada since she was 6 years old)

CULTURAL/RELIGIOUS: PRACTICES/OBSERVANCES: Christianity

PREVIOUS LIVING ARRANGEMENTS: House / Apartment

PREVIOUS OCCUPATIONS/ROLES: Retired IT Project Manager

FAMILY MEMBERS/FRIENDS/VISITORS: Mother

REASON/S FOR BEING IN FACILITY: Client had a post-operative complication (messed up knee replacement surgery) so she was transferred to 5East rehab unit.

LENGTH OF TIME IN FACILITY: Client was admitted to SHN-Centenary 5East Rehab Unit on 09/01/2026 and was discharged on 06/02/2026.

ABILITY TO SEE/HEAR/SENSE OF TOUCH/TASTE/SMELL: All senses well. No impairments noted.

ABILITY TO COMMUNICATE/DEVICES/APPROACH: Client communicates in English, uses a cane to move with no assist needed.

AWARENESS OF SURROUNDINGS/PLACE/OTHER PERSONS/SAFETY: Client is alert and oriented with no assist required. She takes care of her own safety. She is fully independent. Any additional safety measure should be as in the care plan.

PRESENT FEELINGS & CONCERNS: Client is very cheerful and always smiling with a very good mood every time someone goes to visit. No concerns noted

RE: SELF/CONDITION/PLACEMENT/FAMILY/ACTIVITIES/LIFE/COMMUNITY/

etc.

Client lives with her mother. She never married so there was no one who came to visit her. She takes care of her mother. She doesn't have any pain or discomfort, and she was in a elevated level of cheerful mood due to her discharge.

OVERALL APPEARANCE & ABILITIES/LIMITATIONS TO FUNCTION:

Client is fully independent and needs no assistance except for the small chores like throwing garbage, helping with providing water and food outside of the mealtime, etc. She is very clean and likes to keep the surrounding clean. Except the left leg she doesn't have any limitations.

Data Sheet for Client 3

GUIDE – RESIDENT/CLIENT INFORMATION AND NEEDS

The Information can be gathered from the patient/resident/client, family, Care Partner, Faculty Instructor	
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AGE: 79 Y.O. (F) STAGE OF GROWTH & DEVELOPMENT: Late

Adulthood

BIRTHPLACE: Toronto, ON

ETHNIC/CULTURAL BACKGROUND: Caucasian

CULTURAL/RELIGIOUS: PRACTICES/OBSERVANCES: Anglican (Church of England)

PREVIOUS LIVING ARRANGEMENTS: House/ Apartment

PREVIOUS OCCUPATIONS/ROLES: Retired PT (Physiotherapist) / OT
(Occupational Therapist)

FAMILY MEMBERS/FRIENDS/VISITORS: Husband, Daughter, Son-in-law,
Grand-daughters (two)

REASON/S FOR BEING IN FACILITY: Client was transferred to the 5East -
Rehab Unit for management of post-sepsis deconditioning and functional
decline in the context of multiple chronic medical, neurological, and
oncologic comorbidities.

LENGTH OF TIME IN FACILITY: Client was admitted on 07/02/2026 and stayed 5 days on the unit. She was discharged on 12/02/2026

ABILITY TO SEE/HEAR/SENSE OF TOUCH/TASTE/SMELL: Client has a severe ear pain, and it is interfering with her hearing. No other impairments noted.

ABILITY TO COMMUNICATE/DEVICES/APPROACH: Client speaks clear English and uses a walker to commute with minimum assist needed.

AWARENESS OF SURROUNDINGS/PLACE/OTHER PERSONS/SAFETY: Client is usually aware of her surroundings but due to pain clouding her judgement, she is unable to focus on anything.

PRESENT FEELINGS & CONCERNS: Even though there is pain she is always happy and smiling. She is a little more on the talkative side and love to just talk and talk. When she talks she thinks a lot (remembering things) and when

there is a sharp and sudden pain (usually 2-4 seconds) she forgets what she was talking about.

RE: SELF/CONDITION/PLACEMENT/FAMILY/ACTIVITIES/LIFE/COMMUNITY/
etc.

Client with a history of CKD, Hypertension and Uterine cancer admitted due to principal problem of sepsis and for management of post-sepsis deconditioning and functional decline. Client has family members taking care of her. Her husband often visits her. She has a good relationship with her family.

OVERALL APPEARANCE & ABILITIES/LIMITATIONS TO FUNCTION:

Client is weak and uses walker to move. She is neat and clean and like to read and talk to someone a lot. She has a sharp acute ear pain from time to time which lasts about 2-4 seconds. No other impairments noted.

SPECIFIC NEEDS OF THE CLIENT

HOW MET/ SPECIFIC ASSISTANCE REQUIRED/PREFERENCES

WHAT - WHEN - WHERE - HOW'

**Students to complete the needs that relate to their client weekly*

1. SAFETY NEEDS (PRECAUTIONS/ASSISTIVE DEVICES) (Specific/when, where, how):

Client 1: C.K. is at moderate-to-high fall risk due to generalized weakness, congestive heart failure, arthritis, and fatigue. Requires walker assistance for mobility and one-person assist for transfers and toileting. Safety precautions must be maintained at all times with the bed in lowest position, brakes locked, call bell within reach, and supervision during ambulation and showering.

Client 2: C.O.B. is independent with mobility and ADLs following knee surgery recovery but continues to use a cane for stability. Requires a clutter-free environment, reachable personal belongings, and routine safety checks to prevent post-operative instability or accidental falls.

Client 3: D.L.N. has weakness following sepsis and experiences sudden sharp ear pain which may briefly affect concentration and balance. Requires walker

use, standby assistance for transfers, and supervision during movement to prevent sudden instability during pain episodes.

PSW Role for Safety Needs Identify:

Client 1: Maintain safe environment, assist with transfers, ensure walker use, and report weakness or instability immediately (*Wilk, Sorrentino, & Remmert, 2022, pp. 321–323*).

Client 2: Maintain clutter-free environment, confirm safe cane use, and report any mobility change (*Wilk, Sorrentino, & Remmert, 2022, pp. 321–323*).

Client 3: Supervise transfers, monitor sudden pain episodes affecting safety, and report immediately (*Wilk, Sorrentino, & Remmert, 2022, pp. 321–323*).

2, MOBILITY/POSITIONING /EXERCISE/ AND REST (Including assistive devices/wheelchair/lifting/transfer/ambulation/repositioning/active ROM):

Client 1: C.K. requires one-person assistance for transfers, repositioning in bed, and support during short ambulation using walker. Needs frequent rest periods due to fatigue and cardiac condition.

Client 2: C.O.B. is fully independent with bed mobility, transfers, and walking using a cane. She requires only minimal supervision for longer distances to avoid knee strain.

Client 3: D.L.N. requires standby to minimal assist for transfers and walker ambulation due to post-sepsis weakness and intermittent pain affecting endurance.

PSW Role for Mobility Needs Identify:

Client 1: Assist with repositioning, safe transfers, and walker support using proper body mechanics (*Wilk, Sorrentino, & Remmert, 2022, pp. 315–318*).

Client 2: Encourage independent mobility while observing for fatigue or instability (*Wilk, Sorrentino, & Remmert, 2022, pp. 315–318*).

Client 3: Assist with transfers and monitor tolerance to activity (*Wilk, Sorrentino, & Remmert, 2022, pp. 315–318*).

3. HYGIENE: SKIN CARE - REPOSITIONING/CARE TO PRESSURE AREAS:
(Observations-abnormalities/ frequency, type, positioning aids, appearance of all pressure areas, care to pressure areas/skin protective care):

Client 1: C.K. requires assistance with showering, perineal care, and oral hygiene due to weakness. Needs regular skin checks and repositioning to prevent pressure injury.

Client 2: C.O.B. performs hygiene independently but prefers a clean setup and organized environment. Assistance only required if supplies or setup needed.

Client 3: D.L.N. requires supervision or partial assistance with hygiene depending on fatigue level and comfort. She requires monitoring of skin when transferred to chair sitting and decreased endurance.

PSW Role for Hygiene Needs Identify:

Client 1: Provide full hygiene assistance, monitor skin condition, and report redness (*Wilk, Sorrentino, & Remmert, 2022, pp. 324–327*).

Client 2: Provide hygiene setup if requested and ensure safety (*Wilk, Sorrentino, & Remmert, 2022, pp. 324–327*).

Client 3: Assist hygiene as required and observe skin integrity (*Wilk, Sorrentino, & Remmert, 2022, pp. 324–327*).

4. FOODS/FLUIDS/DIET/SPECIAL DIET/SNACKS/USUAL INTAKE PATTERN/APPETITE (Ability to swallow-solids/liquids/ methods of assistance/positioning/cultural needs preferences/frequency/amount/type of fluids/preferences; intake & output measurement)

Client 1: C.K. requires upright positioning before meals, monitoring for fatigue during eating, and encouragement for adequate hydration due to cardiac condition.

Client 2: C.O.B. eats independently with good appetite and requires only meal setup if requested and occasional intake documentation.

Client 3: D.L.N. eats independently but requires monitoring for fatigue and reminders for fluid intake due to distraction from pain.

PSW Role for Nutrition Needs Identify:

Client 1: Position upright, monitor tolerance, and record intake (*Wilk, Sorrentino, & Remmert, 2022, pp. 336–339*).

Client 2: Provide meal setup if requested and document intake when required (*Wilk, Sorrentino, & Remmert, 2022, pp. 336–339*).

Client 3: Observe eating tolerance and encourage fluids (*Wilk, Sorrentino, & Remmert, 2022, pp. 336–339*).

5. NEED FOR COMFORT (Relief of pain/discomfort/distress/comfort measures):

Client 1: C.K. experiences fatigue and general discomfort. So, she requires positioning with pillows and scheduled rest.

Client 2: C.O.B. normally is pain-free but may occasionally have mild leg discomfort requiring rest positioning.

Client 3: D.L.N experiences sudden sharp ear pain often requiring observation, reporting, reassurance, and comfort positioning.

PSW Role for Comfort Needs Identify:

Client 1: Reposition for comfort and report distress (*Wilk, Sorrentino, & Remmert, 2022, pp. 340–342*).

Client 2: Observe for discomfort and report changes (*Wilk, Sorrentino, & Remmert, 2022, pp. 340–342*).

Client 3: Monitor pain episodes and report immediately (*Wilk, Sorrentino, & Remmert, 2022, pp. 340–342*).

6. NEED FOR AIR/BREATHING/OXYGEN: TPR (Previous range/present/
shortness of breath/cough - observations; colour face lips/nails/feet: - colour
& temperature/edema /positioning e.g. extremities; positions for breathing;/
oxygen safety precautions/sputum specimen):

Client 1: C.K.'s CHF requires monitoring for shortness of breath, fatigue, edema, and activity intolerance.

Client 2: C.O.B. has stable breathing with no respiratory concern.

Client 3: D.L.N.'s bronchiectasis history requires observation of breathing pattern and fatigue.

PSW Role for Breathing Needs Identify:

Client 1: Observe breathing difficulty and report immediately (*Wilk, Sorrentino, & Remmert, 2022, pp. 347–352*).

Client 2: Observe breathing during activity (*Wilk, Sorrentino, & Remmert, 2022, pp. 347–352*).

Client 3: Monitor respirations and assist positioning for comfort (*Wilk, Sorrentino, & Remmert, 2022, pp. 347–352*).

7. ELIMINATION NEEDS-URINARY ELIMINATION PATTERN: frequency, observations-amount; colour; odour; clarity; incontinence; assistance/what used to encourage/place/devices - catheter, etc. Urine specimen/Diabetic Urine testing):

Client 1: C.K. requires assistance with toileting and brief checks due to weakness.

Client 2: C.O.B. is independent in toileting with normal pattern.

Client 3: D.L.N. may require assistance reaching washroom depending on the level of strength and pain she has currently.

PSW Role for Urinary Needs Identify:

Client 1: Assist toileting and observe urine characteristics (*Wilk, Sorrentino, & Remmert, 2022, pp. 356–358*).

Client 2: Maintain privacy and observe pattern (*Wilk, Sorrentino, & Remmert, 2022, pp. 356–358*).

Client 3: Assist toileting as needed and report abnormalities (*Wilk, Sorrentino, & Remmert, 2022, pp. 356–358*).

8. ELIMINATION NEEDS-BOWEL ELIMINATION PATTERN: frequency, observations - amount; colour; consistency; other/incontinence/assistance/ place/ devices - ostomy; specimens):

Client 1: C.K. needs monitoring for constipation due to decreased mobility.

Client 2: C.O.B. is independent with all bowel elimination.

Client 3: D.L.N. requires observation for bowel regularity due to illness and reduced activity.

PSW Role for Bowel Needs Identify:

Client 1: Observe bowel movement and report constipation (*Wilk, Sorrentino, & Remmert, 2022, pp. 358–360*).

Client 2: Observe and report changes (*Wilk, Sorrentino, & Remmert, 2022, pp. 358–360*).

Client 3: Assist as required and document findings (*Wilk, Sorrentino, & Remmert, 2022, pp. 358–360*).

9. REHABILITATION/SOCIAL INTERACTION/LEISURE & STIMULATION

NEEDS/INTERESTS/PRACTICES-NEEDS FOR EMOTIONAL SUPPORT

(Identify feelings/ moods/motivation/coping methods/goals/self-concept/adjustment to changes/and specific type of supportive/communication /assistance required):

Client 1: C.K. benefits from encouragement and emotional reassurance. Her brother's visits influence her mood positively.

Client 2: C.O.B. is cheerful and motivated for discharge. Her independence should be respected and encouraged.

Client 3: D.L.N. is very talkative and enjoys conversation. She requires patience during pain interruptions.

PSW Role for Emotional Needs Identify:

Client 1: Provide supportive communication and observe mood (*Wilk, Sorrentino, & Remmert, 2022, pp. 299–302*).

Client 2: Encourage independence and respectful interaction (*Wilk, Sorrentino, & Remmert, 2022, pp. 299–302*).

Client 3: Provide active listening and reassurance (*Wilk, Sorrentino, & Remmert, 2022, pp. 299–302*).

10. ADDITIONAL TREATMENTS/SUPPORT ACTIVITIES (WITHIN PSW ROLE)

FOR CLIENT:

Client 1: C.K. requires assistance with ADLs, intake documentation, safety monitoring, and fatigue observation.

Client 2: C.O.B. requires only environmental support and occasional small assistance.

Client 3: D.L.N. requires monitoring of pain episodes, transfer safety, and functional changes.

PSW Role for Additional Needs Identify:

Client 1: Document care and report condition changes (*Wilk, Sorrentino, & Remmert, 2022, pp. 294–296*).

Client 2: Assist within scope and maintain documentation (*Wilk, Sorrentino, & Remmert, 2022, pp. 294–296*).

Client 3: Observe status changes and communicate with nurse (*Wilk, Sorrentino, & Remmert, 2022, pp. 294–296*).

Reference

Sorrentino, S. A., Remmert, L. N., & Wilk, M. J. (2022). *Sorrentino's Canadian textbook for the support worker* (5th ed.). Elsevier.